

HAROLD JAMES WHITFIELD – ENCOUNTER NOTE

(SYNTHETIC / DE-IDENTIFIED – FOR DEMONSTRATION PURPOSES ONLY)

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MOUNTAINVIEW INTERNAL MEDICINE ASSOCIATES

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PATIENT: Whitfield, Harold J. MRN: 00294817

DATE OF BIRTH: 08/14/1959 (Age: 65) SEX: Male

INSURANCE: BlueCross BlueShield (Federal Employee Plan)

Member ID: XYZ-900284718 Group: 0047-G

ENCOUNTER DATE: 03/22/2024 PCP: Dr. Anita Ramachandran, MD

ENCOUNTER TYPE: Office Visit – Established Patient (E/M Level 4)

REFERRING PHY: Self-referred

DICTATED: 03/22/2024 at 16:35 SIGNED: 03/22/2024 at 18:02

CHIEF COMPLAINT (CC)

Patient presents with a 3-week history of intermittent chest pain, worsening shortness of breath on exertion, and poorly controlled blood sugars. He also notes new-onset bilateral lower extremity edema and fatigue.

HISTORY OF PRESENT ILLNESS (HPI)

Harold Whitfield is a 65-year-old male with a longstanding history of Type 2 Diabetes Mellitus (T2DM) and essential hypertension who presents today for a follow-up visit. He reports his blood glucose readings at home have been consistently elevated over the past three weeks, ranging from 280 to 360 mg/dL fasting, despite compliance with his insulin regimen. He attributes this deterioration to recent dietary indiscretions associated with a family gathering.

Additionally, the patient reports new-onset chest pain that he describes as a "pressure-like" sensation in the mid-sternal area, radiating to the left arm, occurring primarily with exertion (e.g., walking two city blocks) and relieved with rest. He denies palpitations, syncope, or diaphoresis accompanying the chest pain episodes. He rates the pain 5-6/10 at peak.

The patient also notes progressive dyspnea on exertion (DOE) over the past two weeks, now limiting him to less than one flight of stairs. He denies orthopnea or paroxysmal nocturnal dyspnea (PND). He has noticed bilateral ankle swelling that is worse in the evenings.

Review of Systems (ROS):

[+] Chest pressure/pain, dyspnea on exertion, bilateral leg edema, fatigue, increased thirst (polydipsia), increased urination (polyuria), blurred vision (intermittent), numbness/tingling in bilateral feet (chronic).
[-] Fever, chills, nausea/vomiting, abdominal pain, cough, hemoptysis, syncope, palpitations, diarrhea, hematuria.

PAST MEDICAL HISTORY (PMH)

1. Type 2 Diabetes Mellitus (T2DM) – Diagnosed 2007, on insulin since 2019.
Last HbA1c: 10.2% (Result date: 01/10/2024). Target: <7.0%.
2. Essential Hypertension – Diagnosed 2005. Currently suboptimally controlled.
Last BP on record: 158/96 mmHg (Visit date: 01/10/2024).
3. Hyperlipidemia (Mixed dyslipidemia) – Diagnosed 2010.
Last LDL: 142 mg/dL (Result date: 01/10/2024). Target: <70 mg/dL.
4. Peripheral Neuropathy (Diabetic) – Bilateral lower extremities. Dx: 2020.
5. Chronic Kidney Disease (CKD) Stage 3a – eGFR: 52 mL/min/1.73m² (01/2024).
6. Obesity – BMI: 34.2 kg/m². (Weight: 218 lbs, Height: 5'10").
7. Obstructive Sleep Apnea (OSA) – Diagnosed 2021. On CPAP therapy (reported poor compliance, using 2-3 nights per week).

SURGICAL HISTORY

- Appendectomy (1988, uncomplicated)
- Right knee arthroscopy for meniscal repair (2015)

FAMILY HISTORY (FH)

- Father: Deceased at age 70, acute myocardial infarction.
- Mother: Alive, age 88, Alzheimer's disease, T2DM.
- Brother: Living, age 63, coronary artery disease (stents placed 2021), T2DM.
- Sister: Living, age 60, breast cancer (in remission).

SOCIAL HISTORY (SH)

- Tobacco: Former smoker; 20 pack-year history. Quit 2010.
- Alcohol: Social drinker; 2-3 beers/week. Denies binge drinking.
- Illicit Drugs: Denies.
- Occupation: Retired postal worker (30 years). Now sedentary lifestyle.
- Living Situation: Married, lives with wife. Two adult children.
- Exercise: Minimal. Denies structured exercise program.
- Diet: High-carbohydrate; poor adherence to diabetic diet.

CURRENT MEDICATIONS (Rx)

1. Insulin Glargine (Lantus) U-100 – 42 units SQ QHS (bedtime).

2. Insulin Lispro (Humalog) U-100 – Sliding scale, SQ with meals.
 3. Lisinopril 10 mg – 1 tab PO QDay (hypertension/CKD).
 4. Amlodipine 5 mg – 1 tab PO QDay (hypertension).
 5. Atorvastatin 40 mg – 1 tab PO QHS (hyperlipidemia).
 6. Aspirin 81 mg (enteric-coated) – 1 tab PO QDay (cardioprotective).
 7. Gabapentin 300 mg – 1 cap PO TID (diabetic neuropathy).
 8. Metformin 500 mg – 1 tab PO BID (T2DM adjunct).
- ** NOTE: Metformin use with eGFR 52 – borderline; requires monitoring.
9. Omeprazole 20 mg – 1 cap PO QDay (GI prophylaxis).
 10. Vitamin D3 2000 IU – 1 cap PO QDay (supplementation).

ALLERGIES

- Penicillin: Rash (Moderate). Last reaction: 1998.
- Sulfa drugs: GI upset (Mild).
- Contrast dye: No documented allergy; pre-medication protocol PRN.
- NKDA to latex.

PHYSICAL EXAMINATION

VITAL SIGNS:

Blood Pressure: 162/98 mmHg (Left arm, seated)

Heart Rate: 88 bpm (Regular)

Respiratory Rate: 18 breaths/min

Temperature: 98.4°F (36.9°C) – Oral

SpO2: 96% on room air

Weight: 218.4 lbs (99.1 kg)

BMI: 34.2 kg/m²

GENERAL: Alert, oriented x3. Well-nourished, mildly obese male. No acute distress. Mildly diaphoretic.

HEAD/EYES/EARS/NOSE/THROAT (HEENT): Normocephalic, atraumatic. Pupils equal, round, and reactive to light (PERRL). Oropharynx clear. No lymphadenopathy. Fundoscopic exam deferred to ophthalmology referral.

CARDIOVASCULAR: Regular rate and rhythm. S1 and S2 present. A soft S3 gallop is appreciated at the apex. No murmurs or rubs heard. PMI laterally displaced (5th ICS, mid-axillary line – suggestive of cardiomegaly).

PULMONARY: Mild bibasilar crackles (rales) on auscultation. No wheeze or rhonchi. Percussion: dull at bilateral lung bases.

ABDOMEN: Soft, non-tender, non-distended. Bowel sounds active x4. No hepatosplenomegaly. No abdominal bruits.

EXTREMITIES: 2+ pitting edema bilateral ankles and lower legs (to mid-calf).

NEUROLOGICAL: Alert and oriented. Cranial nerves II-XII grossly intact. Monofilament testing (10g Semmes-Weinstein): Absent protective sensation bilaterally in plantar surface and all toes (consistent with peripheral neuropathy). Vibration sense decreased bilateral feet.

LABORATORY RESULTS (IN-OFFICE / RECENT)

- Random Plasma Glucose: 334 mg/dL [Critical High: >250] ■
- Urine Dipstick: 3+ glucose, 2+ protein

- HbA1c: 10.2% [Goal: <7.0%] ■ CRITICALLY HIGH
- eGFR (CKD-EPI): 52 mL/min [CKD Stage 3a – Monitor closely]
- Serum Creatinine: 1.42 mg/dL
- BUN: 24 mg/dL
- Potassium: 4.1 mEq/L
- Sodium: 138 mEq/L
- LDL Cholesterol: 142 mg/dL [Goal: <70 mg/dL for DM] ■ HIGH
- HDL Cholesterol: 38 mg/dL [Low – cardiovascular risk factor]
- Triglycerides: 285 mg/dL [High] ■
- **AST/ALT: 32/28 U/L [WNL]**
- TSH: 2.1 mIU/L [WNL]
- Microalbumin/Creatinine Ratio: 185 mg/g [Moderately Increased – Diabetic Nephropathy]

- 12-Lead ECG: Ordered (results pending at time of dictation)
- BNP (Brain Natriuretic Peptide): Ordered
- CMP, CBC, Lipid Panel: Ordered (fasting, next AM)
- Chest X-ray (PA/Lateral): Ordered

CLINICAL DECISION SUPPORT: This assessment reflects independent clinical judgment. Recommendations follow ADA Standards of Medical Care in Diabetes 2024 and ACC/AHA Cardiovascular Guidelines.

[REDACTED]

PROBLEM 1: UNCONTROLLED TYPE 2 DIABETES MELLITUS [ICD-10: E11.65]

Assessment: Patient's HbA1c of 10.2% represents critically uncontrolled T2DM (target <7.0%). Likely exacerbated by dietary non-compliance, physical inactivity, subtherapeutic insulin dosing, and physiological stress.

Note: Hyperglycemic hyperosmolar state (HHS) risk given glucose of 334.

Plan:

- Increase Insulin Glargine from 42 units to 52 units SQ QHS.
- Continue Insulin Lispro sliding scale; consider increasing correction factor.
- Refer to Certified Diabetes Care and Education Specialist (CDCES).
- Reinforce Medical Nutrition Therapy (MNT); low-carbohydrate dietary guidance.
- Repeat HbA1c in 3 months.
- Consider adding GLP-1 receptor agonist (e.g., semaglutide) at follow-up if glucose not controlled and CKD stage permits.
- Hold Metformin reassessment pending renal function review (eGFR 52).

PROBLEM 2: NEW-ONSET CHEST PAIN – RULE OUT ACUTE CORONARY SYNDROME (ACS)

[Suspected: Unstable Angina or NSTEMI – ICD-10: I20.0 / R07.9]

Assessment: Exertional pressure-type chest pain radiating to the left arm in a 65-year-old male with T2DM, hypertension, hyperlipidemia, obesity, family history of MI, and prior smoking history is HIGHLY CONCERNING for obstructive coronary artery disease (CAD) or ACS. The constellation of symptoms warrants urgent cardiac evaluation. A "silent MI" in the context of diabetic autonomic neuropathy cannot be excluded.

Plan:

- ****URGENT****: Obtain 12-Lead ECG now (stat read). If ST-elevation or LBBB: activate emergency cardiac protocol immediately.
- Obtain troponin I/T (high-sensitivity) serially (0h, 3h).
- BNP ordered – evaluate for concurrent HF.
- Chest X-ray to assess cardiac silhouette and pulmonary vascular markings.
- Cardiology URGENT referral placed (Dr. Marcus Chen, MD – notified by phone).
- Do NOT discharge patient without cardiology clearance today.
- Aspirin 325 mg loading dose: Administer now (patient already on 81mg ASA).
- NPO status pending procedural decisions.

PROBLEM 3: SUSPECTED NEW-ONSET HEART FAILURE (Systolic vs. Diastolic)

[ICD-10: I50.9 – Pending confirmation]

Assessment: Clinical findings – bilateral basibasal rales, bilateral pitting edema 2+, laterally displaced PMI, and an S3 gallop – are consistent with new-onset heart failure, possibly ischemic in etiology given the concurrent chest pain presentation. The displaced PMI suggests left ventricular enlargement.

Plan:

- BNP result pending; if elevated (>400 pg/mL), treat as decompensated HF.
- Echocardiogram (transthoracic – TTE) ordered to assess EF and wall motion.
- Consider IV furosemide if significant volume overload confirmed.
- Fluid restriction: 1.5L/day counselling provided.
- Daily weight monitoring; return if weight gain >3 lbs in 24h.
- Cardiology to guide definitive HF management (GDMT initiation as appropriate).

PROBLEM 4: ESSENTIAL HYPERTENSION – SUBOPTIMALLY CONTROLLED [ICD-10: I10]

Assessment: BP 162/98 mmHg today despite dual antihypertensive therapy. Suboptimal control likely contributing to both CKD progression and increased cardiac strain. Target BP: <130/80 mmHg (ADA/ACC 2024 guideline).

Plan:

- Increase Lisinopril from 10 mg to 20 mg PO QDay (after confirming stable K+).
- Consider adding a thiazide diuretic (chlorthalidone 25 mg) if no response at next visit.
- DASH diet counselling reinforced.
- Home BP monitoring: twice daily readings log requested.

PROBLEM 5: HYPERLIPIDEMIA – INADEQUATELY TREATED [ICD-10: E78.5]

Assessment: LDL 142 mg/dL is critically above the <70 mg/dL target for a diabetic patient with multiple cardiovascular risk factors. Current statin dose (Atorvastatin 40 mg) is insufficient. ASCVD risk is very high.

Plan:

- Uptitrate Atorvastatin from 40 mg to 80 mg QHS (maximum intensity statin).

- If LDL remains >70 mg/dL at 3-month recheck: initiate ezetimibe 10 mg/day.
- PCSK9 inhibitor consideration deferred pending cardiology evaluation.

PROBLEM 6: DIABETIC PERIPHERAL NEUROPATHY – PROGRESSING [ICD-10: E11.40]

Assessment: Loss of protective sensation bilaterally on monofilament testing confirms high-risk foot status. Capillary refill >3 sec suggests concurrent peripheral artery disease (PAD) – evaluation warranted.

Plan:

- Podiatry referral placed for comprehensive foot examination.
- Ankle-Brachial Index (ABI) ordered to screen for PAD.
- Continue Gabapentin 300 mg TID; reassess pain control.
- Patient education: daily foot inspection, proper footwear.

PROBLEM 7: CHRONIC KIDNEY DISEASE STAGE 3a – DIABETIC NEPHROPATHY [ICD-10: N18.3 / E11.65 with N18.3]

Assessment: Microalbumin/Cr ratio of 185 mg/g confirms significant proteinuria consistent with diabetic nephropathy. eGFR 52. Trend monitoring is critical to prevent progression to Stage 4/5.

Plan:

- Nephrology referral placed for co-management.
- Monitor eGFR and microalbumin quarterly.
- Maintain Lisinopril (ACE-I nephroprotective role).
- Dietary protein restriction: 0.8g/kg/day counselling.
- Review Metformin dosing – hold if eGFR drops <45.
- Consider SGLT2 inhibitor (e.g., empagliflozin) – significant renoprotective and cardioprotective benefit; coordinate with cardiology and nephrology.

PROBLEM 8: OBESITY [ICD-10: E66.09]

Plan:

- Dietary counselling re: caloric restriction.
- Refer to Medical Weight Management clinic.

- GLP-1 agonist (semaglutide) under consideration – dual benefit for T2DM and weight reduction; pending CKD clearance.

PROBLEM 9: OBSTRUCTIVE SLEEP APNEA – POOR CPAP COMPLIANCE [ICD-10: G47.33]

Plan:

PROBLEM 10: PREVENTIVE CARE GAPS / MISSING ITEMS (FLAGGED BY CARE COORDINATOR)

- Overdue: Dilated eye exam (last documented: 2022 – OVERDUE)
- Overdue: Dental exam (last documented: unknown)
- Overdue: Colorectal cancer screening (no colonoscopy on file – age 65)
- Missing: Pneumococcal (PPSV23/PCV20) vaccination – immunocompromised (DM)
- Missing: Annual influenza vaccine (current season not documented)

DISPOSITION

Patient is NOT cleared for discharge today. Urgent cardiology evaluation has been requested and is pending. Patient and wife were counselled on the urgency of today's findings, specifically the cardiac symptomatology requiring immediate investigation.

1. ECG and troponin results – stat.
2. Cardiology bedside evaluation today.
3. Possible hospital admission for cardiac monitoring (telemetry).
4. If safe for outpatient: follow-up in 48 hours.

ELECTRONICALLY SIGNED BY:

NPI: 1234567890 | DEA: AR1234567

Signed: 03/22/2024 at 18:02 EST

***** IMPORTANT NOTICE *****

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